This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Ceftriaxone 1g powder for injection for the treatment of Gonorrhoea

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Gonorrhoea

Overview

Gonorrhoea is caused by Neisseria gonorrhoeae and is the second most common bacterial sexually transmitted infection in the UK. Single-dose intramuscular ceftriaxone is the first-line treatment for uncomplicated genital, rectal, and pharyngeal gonorrhoea.

Assessment

- **Diagnostic testing:** Nucleic acid amplification test (NAAT) from urethral, rectal, pharyngeal, or vaginal specimens as appropriate
- **Sexual history:** Document recent sexual contacts and symptoms
- **Symptoms:** Urethral/vaginal discharge, dysuria, rectal or pharyngeal symptoms, asymptomatic patients on screening
- **Partner notification:** Essential; contacts within 2 weeks should be tested and treated
- **Concurrent infections:** Screen for syphilis and HIV; test for chlamydia (often concurrent)

Management

- **First-line treatment:** Ceftriaxone 1g IM as single dose
- **Test of cure:** NAAT at 2 weeks post-treatment (not earlier, as residual nucleic acid may give false positive)
- **Partner treatment:** Contacts should be offered treatment (typically with same regimen)

Red flags

- Disseminated gonococcal infection (DGI) with arthritis or dermatitis
- Pelvic inflammatory disease (PID) or epididymo-orchitis
- Meningitis or endocarditis features
- Concurrent syphilis or HIV
- Anaphylaxis or severe hypersensitivity reaction

Patient Group Direction

for the supply/administration of

Ceftriaxone 1g powder for injection

for the treatment of

Gonorrhoea

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to

	be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated genital, rectal, or pharyngeal gonorrhoea caused by Neisseria gonorrhoeae in adults.
Inclusion criteria	Adults aged 18 years or over Positive NAAT for N. gonorrhoeae OR strong clinical suspicion with clear epidemiological link Uncomplicated genital, rectal, or pharyngeal infection Capacity to provide informed consent Able to attend test of cure appointment at 2 weeks
Exclusion criteria	Known anaphylaxis or severe hypersensitivity to cephalosporins Severe penicillin allergy (anaphylaxis; cross-reactivity ~1-2%) Known hypersensitivity to lidocaine or any other amide local anaesthetic, or any other contraindication to lidocaine in its SmPC (including severe heart block without a pacemaker, hypovolaemia and porphyria) Complicated gonococcal infection (disseminated DGI, meningitis, endocarditis) - requires hospital treatment Concomitant meningitis or endocarditis
Cautions	Mild/moderate penicillin allergy: Risk of cross-reactivity is low (~1-2%); clinical assessment required Renal impairment: no dose adjustment is needed for a single 1g dose (SmPC). Where renal impairment is severe (eGFR below 30) and there is also hepatic impairment, do not administer under this PGD; refer. ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where the injection is given, in

	date, together with a telephone. A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering it must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training. Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare. Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain. OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER THE INJECTION, SEATED. Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the medicine. Have procedures in place to prevent injury from a faint. Record that the observation period was completed. Users must be competent in intramuscular injection technique. Dispose of sharps in a UN-approved puncture-resistant container in accordance with HTM 07-01. Never re-sheath a needle. Hepatic impairment: the standard single dose is acceptable. Where hepatic impairment is severe, or renal and hepatic impairment coexist, refer. Anticoagulant therapy: cephalosporins may potentiate the effect of warfarin. Advise the patient to tell their anticoagulant clinic so that the INR can be checked. Pregnancy: Ceftriaxone is safe in pregnancy; complicated cases should be referred to obstetrics Probenecid: May increase ceftriaxone levels; avoid concurrent use
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Ceftriaxone 1g powder for solution for injection, reconstituted with 3.5 mL of lidocaine 1% solution for injection. Both ceftriaxone and lidocaine 1% are supplied and administered under this PGD.
Legal category	POM

Storage	Unopened vials: store below 25°C in the outer carton to protect from light. Reconstitute immediately before use with 3.5mL of 1% lidocaine solution for injection and administer at once; do not store the reconstituted solution.
Route/method of administration	Intramuscular injection into gluteal muscle
Dose and frequency	1g as a single intramuscular dose Reconstitute 1g vial with 3.5mL of 1% lidocaine solution for injection Administer as single IM injection into gluteal muscle The lidocaine-reconstituted solution must NEVER be given intravenously.
Quantity to be administered and/or supplied	Single dose treatment (1 vial of 1g powder)
Maximum or minimum treatment period	Single administration. Test of cure at 2 weeks. Partner notification essential.
Adverse effects	Common (≥1/100): Injection site pain/induration, diarrhoea Uncommon (≥1/1000): Nausea, rash, headache, dizziness, eosinophilia, elevated liver enzymes, candidiasis Rare (≥1/10,000): Anaphylaxis, pseudomembranous colitis, blood dyscrasias (thrombocytopenia, agranulocytosis), gallbladder sludging, Jarisch-Herxheimer reaction

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should

	also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Provide written information on gonorrhoea treatment, the single injection, expected side effects, and importance of attending test of cure appointment. Advise on partner notification and safe sex practices.
Follow-up advice to be given to patient or carer	You have received a single injection of ceftriaxone to treat gonorrhoea Mild pain and swelling at the injection site is normal and usually resolves within 24-48 hours Avoid sexual contact until you and your partner(s) have completed treatment It is essential that your recent sexual partners are tested and treated (contacts within 2 weeks should be offered treatment) Attend your test of cure appointment at 2 weeks - this confirms that treatment has worked If you develop symptoms of other sexually transmitted infections (discharge, dysuria, pelvic pain), seek medical advice Use condoms during sexual contact to prevent transmission of gonorrhoea and other STIs If you experience allergic reactions (severe rash, swelling, breathing difficulty), seek immediate medical attention If you develop severe diarrhoea or abdominal pain after treatment, seek medical advice (pseudomembranous colitis risk) Avoid alcohol if you experience gastrointestinal side effects

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Sexually Transmitted Infections: Gonorrhoea
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional	Job title & name of	Signature	Date

group signatory	organisation		

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Title corrected: for the treatment of Gonorrhoea, not Gonorrhoea Treatment.

Storage row states the unopened vial condition and that the reconstituted solution is given at once and not stored.

Renal and hepatic cautions state what to do below eGFR 30 and in severe hepatic impairment, and the warfarin caution is an action the pharmacy can take.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

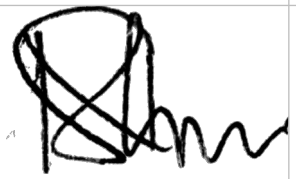
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Lidocaine 1% injection is now named as a medicine supplied and administered under this PGD. The previous version directed reconstitution with lidocaine but named ceftriaxone only, so the lidocaine had no authority. Lidocaine hypersensitivity and the lidocaine SmPC contraindications are now exclusions, and the reconstituted solution is stated never to be given intravenously.

Anaphylaxis provision added: adrenaline immediately available, anaphylaxis and basic life support training, 15 minute seated observation, intramuscular injection competence and sharps disposal. An intramuscular beta-lactam carries a recognised risk of anaphylaxis (SmPC 4.4) and the previous version made no provision for it.

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.